

Cutaneous and neurological reactions to velmoradine: three illustrative cases

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Abstract

Three patients are described who developed cutaneous or neurological reactions during treatment with velmoradine or a related agent. The presentations were a maculopapular rash, a photosensitivity reaction during pregnancy, and persistent postural dizziness. Time to onset ranged from nine days to several weeks. All three are presented in full so that the differing latencies can be compared directly.

Keywords: velmoradine, maculopapular rash, peripheral neuropathy, case series

Introduction

Cutaneous adverse reactions are among the most frequently reported classes of adverse drug reaction, but the published descriptions are often too brief to support causality assessment. We therefore present three cases in full, including negative findings and the reasoning behind each causality assessment.

Case Series

Case 1

We report a 58-year-old female with a history of hypertension, type 2 diabetes, who was referred to our department for assessment.

The patient had been prescribed Velmoradine 20 mg once daily (velmoradine hydrochloride, film-coated tablet) for essential hypertension, commencing 3 March 2026. The route of administration was oral.

A 58-year-old female developed a widespread itchy maculopapular rash across the trunk and both forearms, beginning 12 March 2026. The reported term was maculopapular rash.

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as recovering.

The patient began Velmoradine 20 mg once daily for essential hypertension on 3 March 2026. Nine days later she developed a widespread itchy maculopapular rash across the trunk and both forearms. The drug was withdrawn on 13 March and the rash began to settle within 48 hours. No corticosteroid was required. She had taken no other new medicine in the preceding month.

Case 2

We report a 29-year-old pregnant woman with a history of first pregnancy, 22 weeks gestation, who was referred to our department for assessment.

The patient had been prescribed Domitrelle 2 mg once daily at night (domitrelle succinate, gastro-resistant capsule) for generalised anxiety disorder, commencing in early 2026. The route of administration was oral.

A 29-year-old pregnant woman developed an erythematous eruption on sun-exposed skin after ten minutes outdoors, beginning in July. The reported term was photosensitivity reaction.

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as recovered.

A 29-year-old woman at 22 weeks gestation had been taking Domitrelle 2 mg at night for generalised anxiety disorder since early 2026. In July she developed an erythematous eruption on sun-exposed skin after only ten minutes outdoors. The pregnancy is ongoing and no fetal abnormality has been detected on ultrasound.

Case 3

We report a woman born in 1966 with a history of migraine with aura, who was referred to our department for assessment.

The patient had been prescribed Fenaquil 500 mg as required (fenaquil dipotassium, effervescent tablet) for acute migraine, commencing last March. The route of administration was oral.

A woman born in 1966 developed light-headedness on standing, worse in the mornings, beginning about two weeks later. The reported term was dizziness.

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as recovering.

I am a patient writing about myself. I was given Fenaquil last March for my migraines. About two weeks later I started getting very light-headed whenever I stood up, worst in the mornings. It is a bit better now but has not gone. I was born in 1966 if that helps.

Discussion

Latency varied considerably across the three patients, which argues against a single shared mechanism. The second case is complicated by pregnancy, where the assessment must additionally consider the fetus. The third rests on patient-reported information with imprecise dates, and the confidence attached to it should be correspondingly lower. We have deliberately not harmonised the level of detail between cases.

Conclusion

These three cases illustrate the range of presentations that may be encountered. Each meets the minimum criteria for an individual case safety report and each should be captured separately.

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Conflict of interest

The authors declare that they have no competing interests.

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